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February 20, 2026

James O'Neill
Deputy Secretary
U.S. Department of Health and Human Services
200 Independence Ave. SW
Washington, DC 20201

Thomas Keane, MD
Assistant Secretary for Technology Policy ("ASTP")
U.S. Department of Health & Human Services
200 Independence Ave. SW
Washington, DC 20201
By electronic submission

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care; RIN 0955-AA13

Dear Assistant Secretary Keane and Deputy Secretary O'Neill:

Omada Health, Inc. ("Omada") hereby submits the following comments in response to the above-referenced Request for Information (RFI).

Executive Summary

Omada has been keenly interested in the topic of reimbursing health care professionals for the use of digital health since Omada's founding in 2011. Omada collaborated with the American Medical Association to adopt the first ever CPT code for fully virtual services (0488T), which was established in January 2018. We've published-peer reviewed literature on digital health reimbursement,¹ and we participate actively in industry work groups on the topic.² As discussed below, and as a result of our direct experience and our study of many issues relevant to digital health, incentives, and AI, Omada believes that the most impactful policy change to "incentivize and... promote high-value clinical interventions" (90 F. R. 60109 Dec. 23 2025) (RFI) would be to move more rapidly toward a system of paying for health outcomes instead of paying for time.

ASTP and its sister agency CMS should become very well versed, through wide public input, in:

¹ Kevin G. Volpp, MD, PhD, Stacey Chang, MS, and Sean Duffy, Biggest Challenges to Redesigning Health Care: The real value in health care systems is in human interactions, [NEJM Catalyst](#), December 21, 2017;

Robert Horne and Lucia Savage, Rethinking CMS Coverage And Reimbursement For The Fourth Industrial (AKA Digital) Revolution, *Health Affairs Forefront*; October 23, 2020, [10.1377/forefront.20201021.783035](#);

Robert Horne and Lucia Savage In The Digital Era, Payment Reform Is Key To Shaping A Modern Medicare Program, *Health Affairs Forefront*, February 22, 2021, [10.1377/forefront.20210218.976675](#); and

Sachin Jain MD and Sean Duffy To Integrate Virtual Care, Start By Redefining The 'Visit,' *Health Affairs Forefront*, July 6, 2022, [10.1377/forefront.20220701.584788](#)

² For example, the AMA Value-Based Care Workgroup.

1. the costs of developing and running AI in health care organizations, and
2. the risks of fraud, waste and abuse or other program integrity challenges in connection with the use of AI in clinical practice, as compared to the risks in the use of analog or EHR data (as in the present state).

CMS should develop this understanding using the Administrative Procedures Act to hold listening sessions, take testimony, use Requests for Information, and collect specific information focused on these specific issues to ensure that policy-making about incentivizing the use of AI in clinical practice is not based on casual information collection or potentially unfounded assumptions about the low cost of technology. Responsibly leveraging AI requires significant technical infrastructure, computing power, cybersecurity defenses, and sophisticated diligence from subject matter experts ranging from data scientists to clinicians. To account for this complexity, policy-makers must collect diverse and specific business information from entities who bear these costs today. Only then can policy-makers fairly set payment policies to “massive[ly affect] how health care is delivered in the United States.”

Finally, policy-making should account for the nature of our healthcare system fifteen or more years in the future, not just catch up to the healthcare system we have today.

I. Vision 2040

In 2040, the U.S. will have a healthcare system where AI reduces the time it takes to treat illness and to improve health. We will leverage the digital health foundation we built with the HITECH Act to document the health outcomes that we achieve. AI will be used not only to help us understand the outcomes but also, more importantly, to achieve them. Most ambulatory care will track individuals' health using AI to identify opportunities for intervention, reminiscent of prior data science work to remediate HEDIS gaps in care but occurring in real time instead of a 12-18 month data lag. Even in acute care hospitals, AI will be used effectively to predict where interventions can prevent worsening health or to bend utilization curves differently, helping more people who need acute services to get them at the right time and avoiding wasteful, unnecessary care.

As we move towards 2040, the specific amount of time spent on a particular person's care will become less and less relevant, and the following will be more and more relevant:

- Ensuring health care professionals use AI to accurately document outcomes (good and bad) so that we can pay for outcomes instead of for time
- Understanding the real costs to professionals of using AI effectively to work toward those better outcomes, so that the outcome rates can be set appropriately
- Bringing patients and consumers along by ensuring that they understand how their data contributes to the AI solutions that help them improve their health and stay healthy and solutions enable collaboration towards better health between patients and professionals
- Freeing up the time of professionals to do the more intense, human-necessary care that our professionals are trained for and long to do.

We believe ASTP and other policy-makers should move more rapidly towards value-based and outcomes-based care and that policy decisions should be based on widely collected information about the costs to develop and run an AI-supported health care organization.

II. Background on Omada

Omada is a 15-year-old digital health company that provides health care services centred on human care teams for the prevention of diabetes and other cardiovascular diseases and for the management of diabetes, hypertension and cholesterol (whether as single conditions or comorbid). Since 2020, we also offer video-based tele-physical therapy using licensed physical therapists. In

2024, we launched care tracks within our lifestyle behavior change programs designed to support individuals taking GLP-1s for weight loss.

As we deliver our digital care to members, Omada receives a wide variety of important and helpful data. For example, our programs connect to third-party FDA-approved devices (Class 3, 2, and 1) and leverage computer vision to process real-time biometric information from our members. That data is then displayed in real time to our care professionals in our bespoke systems and to our members themselves through our healthcare mobile applications. That data also forms an important part of our data science and AI work in support of members. Omada has used data science techniques since its founding, and more recently, we have developed several of our own AI-powered features, including generative AI features for member wellness education as well as internal tools that support our clinical staff with summarization or in making clinical recommendations. As of February, 2026, our data stores included 300 million weigh-ins from members in Omada programs.

Even in Omada's early days, we used advanced data science to monitor patterns in our members' data in real time. In 2018, our chief data scientist shared one example at HealthDatapalooza. Our data science techniques discovered that a member had begun tracking food unexpectedly at 4am. The data science team then alerted the member's health coach, who used that observation to start an important conversation about how the member's work schedule had changed and how the member could manage healthy eating despite that schedule change.

More recently, at Omada's 2025 [Horizon Day](#) event, Omada launched OmadaSpark, a generative AI chatbot. OmadaSpark currently has two key functions: to support members with nutrition education and to help members identify their own barriers to a healthier lifestyle. Since Horizon Day, we've also launched other member-facing AI educational tools.

Data science and, increasingly, AI also help us to remain aligned with our customers and members. Omada's business model seeks to align our interests with the interests of our members and the insurance companies and other customers who offer our programs and reimburse Omada for care. In general, our latest pricing models for cardiometabolic programs are based on each member's level of activity in our programs in an effort to tie payments to desired health outcomes. Our view is that, with data science and AI, we can improve our programs' efficiency, improve member engagement and health, and support our care professionals' expertise. Accordingly, we don't charge our customers more for the use of AI; they expect us to use these technologies. We believe that will be an expectation across healthcare by 2040.

III. Detailed Comments on ASTP Questions

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Much of the public dialogue on barriers to AI uptake in clinical practice focuses on (1) liability for potential harm caused by AI and (2) privacy issues.

As a healthcare company, Omada does not believe that the potential for mistakes in care alone must be a barrier to AI uptake. In all care environments, care must meet minimum quality and safety standards to reduce mistakes and manage liability, whether through tools like formal education or software. The use of AI is no different. While AI tools present new factors to navigate, healthcare

has navigated new factors for many earlier innovations - from accuracy concerns about fax machines using thermal paper to print outputs to the use of Class 3 FDA-approved devices. Safety and allocation of responsibility and liability are important issues, but Omada does not see a federal policy role in influencing the allocation of that liability amongst business counterparts.

In the area of privacy, ASTP could be very helpful. For example, many - including AI developers - may not appreciate that AI tools are likely not “conduits” exempt from HIPAA’s business associate rules.³ Similarly, AI developers may not understand that HIPAA business associates can use PHI only for the healthcare operations of their covered entity customers (and not for their own, separate business operations). Therefore, health care providers and other HIPAA covered entities may not have access to adequate AI tools that meet HIPAA requirements. Today, there is an educational role for ASTP and OCR in particular (and HHS in general) to contextualize the HIPAA rules for the advent of AI, much like those agencies did with the passage of the HITECH Act but related to EHRs and cloud computing.⁴

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Many organizations, including large, multi-site academic health institutions,⁵ are assessing the novel issues that arise from AI and developing their own governance structures to meet their needs and environments. Omada has taken a cross-functional approach to develop its own governance model, leveraging the information security concept of a “threat model” to create [a model for assessing AI risk in developing AI features for healthcare](#), which Omada published as an open-source tool of sorts. Other organizations have looked to establish or leverage outside governance certifications. Even in traditional healthcare, the now-standard tools of HITRUST and SOC 2 evaluation of security controls, while not federally mandated, have taken root throughout the health care system. As many of the largest players begin to dominate AI development, it can be a challenge for customers to obtain reasonable limitations of liability or indemnities from AI developers, but this issue exists beyond the realm of AI and unfortunately arises with the largest players in any industry. Nevertheless, we believe that many organizations are meeting the challenge of thoughtfully developing governance structures.

Similar to our answer to Questions 1 and 2 above, we believe ASTP may play a role in promoting the development of effective, compliant models for stakeholders to leverage. It may also have an important role in helping to avoid regulatory capture of AI oversight by large incumbents. Any actions by ASTP should bear in mind the rapid evolution of AI and foster the innovation that AI brings, even from newcomers.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Omada frequently answers potential customers’ questions about how our AI solutions are designed and controlled, including how we ensure safety and quality minimums, how we maintain fidelity with clinical standards, and how we protect privacy. In some cases, broadly adopted measures are becoming clear almost as fast as AI features are being developed. While ASTP could accelerate development of appropriate measures by cataloging those measures and fostering common

³ HHS OCR [FAQ on Conduits](#)

⁴ American Restoration and Reinvestment Act section 134303, 42 USC 17933

⁵ University of California UC Health [AI Governance Task Force Report](#), March 2024

definitions and methods across healthcare, we caution against adoption of specific measures via regulation. Measures codified in regulation can become inflexible and rapidly outdated, particularly in rapidly evolving spaces.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

The use of AI presents important risks to consider, as some AI tools may cause harm, exploit users, misuse data, or simply fail to work. Inadequate AI tools can slow the adoption of these much-needed innovations and the efficiencies they can bring to healthcare. Nevertheless, care must be taken to ensure that well-funded and other dominant organizations in the AI space do not control accreditation or certification processes that freeze out smaller, potentially innovative organizations.

Omada's approach has been to rely on fundamental concepts of healthcare compliance, not outside accreditors. For example, the healthcare industry understands baseline concepts of safety and appropriate quality well. Specialty society guidelines inform clinical best practices. Omada's own nutrition education tool, OmadaSpark, is fine-tuned in part with specialty society guidelines and Omada's own clinical practice guides for operationalizing those guidelines. Existing federal policy, including the latest FDA Guidelines on clinical decision support, align with this approach.

We believe that, in the short term, ASTP can best support and foster the adoption of AI in healthcare by ensuring that AI embraces the unique safety, quality, privacy, and security needs of the healthcare system and ensuring developers and health care organizations have aligned understanding of these fundamentals.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Omada believes that developing AI tools - or even acquiring AI technology from other companies - is a multifaceted and cross-functional project most effectively approached collaboratively among those with clinical, legal, regulatory, privacy, information security, and data science expertise.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

We believe an important step in enhancing interoperability and widening the use of AI in healthcare would be to ensure that any digital health care providers that adhere to the same legal, privacy, and security standards as traditional providers are treated in the same regard as traditional providers under TEACA governance rules. Omada has previously shared observations on this topic in its comments to HTI-2.

IV. Conclusion

The Fourth Industrial Revolution in healthcare is here - powered by data science, digital data, and AI in particular. ASTP and CMS policy-making can massively impact how technology-enabled care is delivered. To ensure healthcare can leverage the transformative potential of AI, we believe policy-making should move more rapidly toward value-based and outcomes-based payment models. Appropriate, modern payment policies will incentivize AI use in clinical care. Moreover, policy-making should be well informed about the costs of developing and using AI in clinical practice because leveraging private, secure, and safe AI to produce clinically sound outcomes is a complex endeavor.

Respectfully submitted,

A handwritten signature in cursive script, appearing to read "Lucia C. Savage".

Lucia C. Savage
Chief Privacy & Regulatory Officer
Omada Health, Inc.